

Multidrug-Resistant Organism (MDRO) Management

Policy Number	POL-IC-025	Owner / Department	Infection Prevention & Control
Effective Date	2024-01-10	Version	2.1
Last Revised	2026-02-25	Next Review	2028-02-25
Approved By	Infection Prevention & Control Committee	Applies To	All clinical units, the laboratory, antimicrobial stewardship, and environmental services

1. Purpose

To detect, contain, and prevent transmission of multidrug-resistant organisms, consistent with the CDC/HICPAC Management of Multidrug-Resistant Organisms in Healthcare Settings and the CDC Containment Strategy for novel or targeted resistance.

2. Definitions

- **MDRO** — a microorganism resistant to one or more classes of antimicrobial agents, including MRSA, VRE, ESBL-producing Enterobacterales, and carbapenem-resistant organisms.
- **CRE** — carbapenem-resistant Enterobacterales, a high-consequence, often carbapenemase-producing MDRO.
- **Active surveillance testing** — screening cultures (e.g., nares, rectal) of at-risk patients to detect asymptomatic colonization.
- **Cohorting** — grouping colonized or infected patients (and, where needed, dedicated staff) to limit transmission.

3. Policy and Procedure

1. Place any patient known or suspected to carry an MDRO on Contact Precautions (gown and gloves) in a single room when available; cohort when single rooms are scarce.
2. Perform active surveillance testing for targeted organisms (e.g., CRE, *Candida auris*) on admission for defined high-risk populations and on transfer from high-prevalence facilities.
3. Dedicate noncritical equipment to the patient; clean and disinfect shared equipment and the environment with an EPA-registered agent effective against the organism.
4. Engage Antimicrobial Stewardship for appropriate therapy and de-escalation; notify the laboratory to flag and report targeted organisms promptly.

IMPORTANT — A single clinical or surveillance isolate of a carbapenemase-producing organism or *Candida auris* must be reported to Infection Prevention and the state health department immediately, and the patient placed on Contact Precautions at once — these novel/targeted resistance findings trigger the CDC Containment Strategy, including a point-prevalence survey of contacts.

4. MDRO Classification

Tier	Organisms	Response
Containment	CRE/CP-CRE, <i>C. auris</i> , novel resistance mechanisms.	Immediate isolation, public-health notification, point-prevalence survey.
Endemic	MRSA, VRE, ESBL producers.	Contact Precautions, surveillance, routine reporting.
C. difficile	Toxigenic <i>C. difficile</i> .	Contact Precautions, soap-and-water hand hygiene, sporicidal cleaning.

5. Severity & Reporting

MDRO events follow POL-RM-013. Identification of a containment-tier organism (CP-CRE, *C. auris*), or any MDRO infection progressing to sepsis or death, is SEV-1 — notify the attending provider, Nursing Supervisor, Risk Management, Infection Prevention, and the Administrator-on-Call immediately and within 1 hour, and Infection Prevention notifies the state health department. A new healthcare-associated MDRO infection requiring treatment is SEV-2 (notify within 4 hours). New colonization without infection, or a lapse caught before transmission, is SEV-3 or a Near Miss, reported within 24 hours.

6. Documentation Requirements

- Organism, specimen source, resistance mechanism (if known), and whether colonization or infection.
- Precautions initiated, room/cohort assignment, and surveillance-testing results.
- For containment organisms: public-health notification and point-prevalence survey; assigned severity and all notifications.

7. References

- CDC/HICPAC, Management of Multidrug-Resistant Organisms in Healthcare Settings (2006, with updates).
- CDC, Interim Guidance for a Public Health Response to Contain Novel or Targeted MDROs (Containment Strategy).
- SHEA/IDSA/APIC Compendium strategies for MRSA, CRE, and *C. difficile*.
- Related: POL-RM-013, POL-IC-020, POL-IC-024.